Weekly cetuximab + irinotecan

Supportive-care review sheet · Individualise with the team; not a prescription.

Before treatment

Confirm exact indication, current symptoms, allergies, blood counts, renal/liver function and medicines. Validated extended RAS wild type is required for colorectal cetuximab; it is not the selection test for head/neck radiotherapy. Check minerals, previous infusion reactions, red-meat allergy and tick bites.

Premedication and nausea

Cetuximab antihistamine plus corticosteroid at least one hour before infusion, including subsequent infusions; follow local policy. Antiemetics per NCCP/local moderate-risk protocol. Assess prophylactic atropine suitability; separate delayed-diarrhoea plan. moderate emetogenic risk; provide rescue instructions.

Administration

Cetuximab on Day 1 and Day 8. Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan. Chemotherapy starts no earlier than one hour after cetuximab ends. Weekly first infusion about 120 minutes, subsequent 60 minutes; fortnightly 500 mg/m² over 120 minutes. Observe per source and local policy.

Skin, nerves and minerals

Provide EGFR skin-care plan, sun protection and prescribed treatments. Track severe skin episode number, delay and recovery. Monitor electrolytes during and monthly for two months after cetuximab. Report new or worsening symptoms early.

Bowel symptoms

Provide distinct early cholinergic and delayed-diarrhoea plans. Check atropine suitability, including glaucoma/asthma precautions. Diarrhoea must fully resolve before irinotecan; serious symptoms require urgent assessment.

Access and discharge

No ambulatory fluorouracil pump. Confirm venous access, transport, observation and the next visit.

Medication review

Review interactions and supplements with pharmacy. Check CYP3A modifiers and UGT1A1 risk. Discuss fertility, pregnancy, contraception and breastfeeding for the complete regimen.

Contact and follow-up

Give the guide, passport and contact card. Use teach-back for fever/illness, chest or breathing symptoms, severe bowel symptoms and line problems. New eye symptoms or infected/blistering skin need prompt review.

Missing inputs remain unassessed; independent clinical/pharmacy review pending. RAS selection applies to colorectal cancer; 00207 head/neck radiotherapy is separate. Cetuximab premedication, skin recurrence and post-treatment electrolyte checks. Source conflict: diarrhoea table gives cetuximab 400/300 mg/m² despite weekly 250 mg/m² maintenance. Automated restart dosing is suppressed.

Sources: NCCP 00330 · EMA Erbitux information · NCCP antiemetic guidance